INDIAN SKIN® · WORKSHEET

What keeps switching your melasma on.

You faded it. It came back. You faded it again. That is not a failure of effort — it is a plan aimed at the stain instead of the leak.

How to use this. Work through the three driver lists on the next page and tick what applies to you. Fill in the last page. Bring it to your appointment, or photograph it and send it ahead.

A dermatologist can read your drivers off this sheet in two minutes. It is the difference between a consultation that starts at the beginning and one that starts where you actually are.

WHY THIS MATTERS

Creams, peels and lasers all work on pigment that has **already formed.** None of them switches off what keeps forming it. That is why melasma comes back within weeks of stopping, and why a plan aimed only at fading is a plan that has to run forever.

Melasma is a chronic, relapsing condition — managed the way blood pressure is managed. Anyone promising it will never come back has not mentioned the relapse rate.

Educational information. Not a diagnosis and not a treatment recommendation. Your skin needs to be examined before anything is decided.

DRIVER ONE — LIGHT, AND NOT JUST SUNLIGHT

Melasma reacts to **visible light and heat**, not only UV. Tick anything that is part of your week.

- [ ] I drive, or sit by a window, for an hour or more most days
- [ ] I cook over a flame or tandoor regularly
- [ ] My sunscreen is **untinted** (no iron oxides)
- [ ] I apply sunscreen once in the morning and not again
- [ ] I skip sunscreen indoors, or on cloudy days

DRIVER TWO — HORMONES

The half no cream can reach, and the half most people are never asked about.

- [ ] It started during or after a **pregnancy**
- [ ] I take the **pill**, or have a hormonal IUD
- [ ] I have a **thyroid** condition, or have never been tested
- [ ] I have PCOS, or irregular cycles
- [ ] Someone in my family has the same patches

DRIVER THREE — OVER-TREATING

Strong actives on an irritated barrier make melasma **worse**, because inflammation is itself a pigment trigger.

- [ ] I use a scrub, or acids, most days
- [ ] I have used a skin-lightening cream for months without review
- [ ] I change products every few weeks when one does not work
- [ ] My skin stings, flushes or feels tight after my routine
- [ ] I have had peels while my skin was already irritated

If you ticked anything in this last list, start there. Calming the barrier often does more in six weeks than another active would.

WORTH ASKING ABOUT

Not everyone needs every test. These are the ones that change a melasma plan when they come back abnormal.

- [ ] **Thyroid function** — TSH, and free T4 if TSH is abnormal
- [ ] **Ferritin and haemoglobin** — if you are tired as well as pigmented
- [ ] **Vitamin D**
- [ ] A **review of your contraception**, with your gynaecologist — never stop it on your own
- [ ] A look at every medicine you take, for photosensitising ones

BRING THIS TO YOUR APPOINTMENT

How long have you had it?

What have you already tried, and for how long?

What made it worse? What made it better?

What are you using on your face right now — everything, including soap?

One line is worth more than a photograph. Most people arrive able to describe the patches and unable to describe the fortnight before they appeared. The fortnight is the useful part.